

New proposed size for Depropin Tab PI (Page 1)
Front 160mm W x 220mm H

[DUOPHARMA (M) SDN BHD]

DEPROPIN TABLET 25MG

DESCRIPTION:

DEPROPIN TABLET 25MG: A 7 mm diameter round shaped tablet, apple green in colour with “DUOMA/DUOMA” and “DUO 861” markings.

COMPOSITION:

Each tablet contains Dothiepin Hydrochloride 25 mg.

PHARMACODYNAMICS:

Actions: Tricyclic antidepressant

Pharmacology: Dothiepin is a thionalogue of amitriptyline. In anti-reserpine activity, it is generally equivalent to amitriptyline but less potent than imipramine.

Site and mode of action: The site of action is thought to be in the CNS, but the mechanism by which this drug and all tricyclic antidepressants produce an antidepressant effect is unknown. Dothiepin possesses anticholinergic, antihistamine and central sedative properties. It is postulated that the aetiology of depression is associated with a functional abnormality of one or more of the biogenic amines, particularly the catecholamines, in the brain. The tricyclics inhibit uptake of noradrenaline and 5-hydroxytryptamine from the nerve endings thus increasing their availability at central noradrenergic synapses.

PHARMACOKINETICS:

Absorption: Well absorbed. Animal studies show dothiepin is absorbed from the small intestine. After a single 50 mg oral dose in one patient, a peak level of 20 nanogram / mL was achieved after 3 hours, falling to 10 nanograms / mL at 6 hours. There are substantial inter-individual variations in steady state concentrations. In 10 patients taken 100 mg / day for two weeks, the serum concentration ranged from 18-84 nanogram / mL (mean 41 ± 7 nanogram / mL). After increasing the dose to 175 mg / day for a further two weeks the concentration ranged from 43 to 196 nanogram / mL (mean 96 ± 15 nanogram / mL). Steady state concentrations appear to be reached after 10 to 14 days.

Distribution: Dothiepin is present in low concentrations in breast milk. It crosses the placental and blood brain barriers in animals. Animal studies in the dog or cat show maximal concentration after 24 hours in liver, uveal tract of the eye, lung, kidney, pituitary and thyroid in descending order. In dogs, the tissue / plasma ratio for uveal tract tissue was 257:1.

Protein binding: Unchanged drug is about 84% bound to serum protein.

Metabolism: Dothiepin is metabolized in the liver and in man 12 basic metabolites were found in the urine, the major of which are northiaden sulphoxide and dothiepin sulphoxide. The metabolic pathways are thought to consist of N-demethylation, S-oxidation and glucuronic acid conjugation. There is active enterohepatic circulation in animals but this has not been studied in humans.

Excretion: 71% of a 50 mg labelled dose is excreted in the urine and faeces within 4 days, 56% being by the renal route.

Half-life: The elimination half-life is biphasic; the first phase is 15 hours, the second 56 hours. Mean whole body elimination half-life is 51 hours.

INDICATION:

The treatment of depression of any aetiology including reactive (exogenous) depression associated with anxiety.

RECOMMENDED DOSAGE:

Note: When a satisfactory response has been obtained, dosage should be reduced to the smallest amount necessary to maintain relief of the symptoms of depression. Plasma levels will reach a new steady state 10 to 14 days after each up or down adjustment.

Adults: Commence with 25 mg three times a day for one to two weeks. If response is inadequate the daily dose should be decreased by 25-50 mg at intervals of one to two weeks. The daily dose should not exceed 200 mg. When an effective dose has been established, up to 150 mg of the daily dose may be given as a single night time dose.

Paediatric: Not recommended for use in children under 16 years since safety and efficacy in this age group has not been established.

Elderly: Use adult dosage with care particularly in patients with impaired liver or renal function or cardiovascular disorders.

With impaired liver or renal function: Dothiepin is contraindicated in hepatic failure.

Use with caution and reduce the dosage in patients with impaired liver or renal function since toxic blood levels may develop. Dothiepin is extensively metabolized by the liver, is thought to undergo enterohepatic circulation and its metabolites are excreted in the urine.

ROUTE OF ADMINISTRATION:

Oral

CONTRAINDICATIONS:

Epilepsy; seizure thresholds may be lowered by the drug.

Tricyclic depressants should not be used concomitantly or within 14 days of treatment with MAO inhibitors since the combination may cause cerebral excitation followed by coma and dangerous hyperthermia.

Acute recovery phase following myocardial infarction; tricyclic antidepressants may produce conduction defects and arrhythmias. Hepatic failure (see Dosage with Impaired Liver or Renal Function). Hypersensitivity to dothiepin.

WARNINGS & PRECAUTIONS:

Suicidality in Children and Adolescents

- Antidepressants increase the risk of suicidal thinking and behaviour (suicidality) in children and adolescents with major depressive disorder (MDD) and other psychiatric disorders.
- Anyone considering the use of antidepressants in a child or adolescents for any clinical use must balance the risk of increased suicidality with the clinical need.
- Patients who are started on therapy should be observed closely for clinical worsening, suicidality, or unusual changes in behaviour.
- Families and caregivers should be advised to closely observe the patient and to communicate with the prescriber.
- Depropin Tablet is not approved for use in pediatric patients.

Electroconvulsive therapy: The hazards of ECT may be increased as the drug lowers the convulsive threshold.

Elective surgery: The drug should be withdrawn prior to surgery.

Severe depression: In patients with severe depression, the possibility of suicide using this drug exists and hence they should be closely supervised during early therapy. These patients should not receive large quantities of the drug.

Manic depressive psychosis: The drug may provoke a shift towards the manic phase.

Monoamine Oxidase Inhibitors: Do not prescribe dothiepin concurrently or within 14 days of MAOI's. After withdrawal of MAOI's initiate therapy at low doses and gradually increase to the normal range.

Cardiovascular disorders: Use with caution as the drug may provoke conduction defects and arrhythmias.

Hyperthyroidism or patients being treated with thyroid hormone: Closely supervise these patients as the drug may provoke cardiac arrhythmias or conduction defects.

Glaucoma, urinary retention and concurrent anticholinergic therapy: Dothiepin has an anticholinergic action and can exacerbate glaucoma and urinary retention and potentiate anticholinergics.

Concurrent therapy with sympathomimetic drugs: Tricyclic antidepressants have been reported to produce possibly dangerous potentiation of the effects of sympathomimetic drugs.

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Precautions: Latent schizophrenia may be activated by dothiepin. Paranoid delusions with or without associated hostility may be exaggerated.

Renal or hepatic impairment: Use with care as toxic blood levels may develop.

Ophthalmological examination: Eyes should be examined regularly for visual acuity and colour fields checked during prolonged therapy since the drug or its metabolites may accumulate in the pigmented area of the eye in experimental animals.

Impairment of motor co-ordination: Ability to drive or operate machinery may be impaired as alertness is decreased.

Elderly patients: Use with care as confusional states may occur. Dependency potential is unknown.

Abrupt withdrawal may produce headache, nausea, convulsions and the possibility of thrombotic episodes.

INTERACTION WITH OTHER MEDICAMENTS:

Alcohol: The effect of alcohol may be potentiated by dothiepin.

Other drugs:

Barbiturates: the sedative effect may be potentiated.

Transquillisers and CNS depressants: the sedative effect may be potentiated.

Guanethidine and other adrenergic neurons blocking drugs: the hypertensive effect may be blocked by dothiepin.

Sympathomimetic: the sympathomimetic effect may be dangerously potentiated by dothiepin.

Monoamine oxidase inhibitors: a potentially lethal interaction can occur between MAOI's and tricyclic antidepressants.

Anticholinergics: dothiepin may potentiate their anticholinergic effects.

Antihistamines: may be potentiated.

Food: no information available.

Interference with clinical, laboratory and other tests: no interference reported with laboratory tests.

USE IN PREGNANCY & LACTATION:

Use in Pregnancy: Tricyclic antidepressants have not been shown to be associated with an increased incidence of birth defects. However, there is evidence of interference with central monoamine neurotransmission in rats. Care should be taken that there are sound indications for the use of these antidepressants in pregnancy.

Use in Lactation: Small amounts of dothiepin have been observed in breast milk and its possible effect on the child must be carefully considered if it is necessary to give the drug to nursing mothers. Withdrawal symptoms that can occur on abrupt cessation of tricyclic antidepressant.

SIDE EFFECTS:

Occur in about 30% of patients and may be severe enough to discontinue the drug in 10% of patients.

More common reactions:

Central nervous system, neuromuscular: Drowsiness, dizziness, tremor, extrapyramidal symptoms, confusional states, paraesthesia, alterations to EEG patterns, disorientation.

Anticholinergic: Dry mouth, urinary retention, sweating.

Cardiovascular: Hypotension, tachycardia, arrhythmias, conduction defects, palpitations.

Endocrine: Increased or decreased libido in either sex.

Gastrointestinal: Nausea, vomiting, constipation.

Ocular: Disturbance of accommodation (blurred vision).

Several of the following reactions have not yet been reported with dothiepin but must be borne in mind because of its similarity to other antidepressants.

Less common reactions:

Central nervous system, neuromuscular: Disturbed concentration, delusions, hallucinations, excitement, anxiety, restlessness, insomnia, nightmares, peripheral neuropathy, ataxia, incoordination, seizures, fatigue, headaches.

Anticholinergic: Paralytic ileus

Cardiovascular: Hypertension, myocardial infarction, heart block, stroke.

Endocrine:

Males: Gynaecomastia, testicular swelling, impotence.

Females: Galactorrhoea.

Gastrointestinal: Epigastric distress, abdominal cramps, blank tongue, peculiar taste sensations, parotid swellings, diarrhoea.

Haematological: Bone marrow depression including agranulocytosis, thrombocytopenia, eosinophilia.

Hepatic: Cholestatic jaundice; altered liver function.

Allergic: Skin rash, urticaria, angioneurotic oedema, photosensitisation, skin blisters.

Other: Weight loss, urinary frequency, mydriasis. Increased appetite and weight gain have been reported but it is not known whether it is due to relief of depression or to the drug.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Overdosage: The toxicity of tricyclic antidepressants is attributed mostly to their anticholinergic effects which produce dry mouth, blurred vision, mydriasis, ileus and urinary retention. Common CNS symptoms are agitation, delirium, ataxia, hyperpyrexia, convulsion, respiratory depression and coma. Cardiovascular symptoms include cyanosis, hypotension, shock, tachycardia and cardiac arrhythmias which are often the major cause of death.

Individual response varies, e.g. death has resulted from overdosage with 0.75 to 1 g of dothiepin but recovery has occurred after as much as 2 g. Serious overdosage with tricyclic antidepressants in children occurs more easily with a relatively small total dosage because the dose per weight ratio is higher.

Treatment: Gastric lavage. Induce emesis with Syrup of Ipecacuanha. In the unconscious patient with no cough reflex the lungs should be protected with a cuffed endotracheal tube. As the drug is thought to undergo enterohepatic circulation, repeated aspiration may be benefit by removing the drug and its metabolites excreted into the gut via the bile. Haemodialysis is not recommended as the absorbed drug rapidly enters the tissue compartment and is very slowly eliminated. (Whole body elimination half-life = 51 hours). General support of the circulation and respiration may be required. ECG monitoring is required. Propranolol and xylocaine may be useful in controlling arrhythmias. Convulsions may be controlled with diazepam. Due to their respiratory depressant effects, barbiturates should be avoided, especially if the patient is thought to have been on monoamine oxidase inhibitors or, if barbiturates have been taken in association with the antidepressant in the overdose.

STORAGE CONDITIONS:

Store below 30°C in airtight container. Protect from light. Keep out of reach of children. *Jauhkan daripada kanak-kanak.*

SHELF LIFE:

Please refer to outer package.

PACK SIZE:

Pack in containers of 100

PRODUCT REGISTRATION HOLDER & MANUFACTURER:

DUOPHARMA (M) SDN BHD

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